SLEEP 12

Tesamorelin

Synthetic 44-amino-acid GHRH analogue with a hexenoyl group attached, given by injection



CLASS	STUDIED FOR	HUMAN EVIDENCE	US STATUS
Peptide (GHRH analogue)	HIV belly fat, liver fat, memory	Large trials; FDA approved	FDA approved (Egrifta); sleep off-label

STUDIED FOR

- **HIV belly fat** — HUMAN
- **Liver fat** — HUMAN
- **Memory and thinking** — HUMAN
- **Sleep** — HUMAN

Sleep use is off-label. A 22-person pilot found no significant change in sleep. Two planned sleep trials were withdrawn before enrolling anyone.

EVIDENCE & WHAT'S KNOWN

EVIDENCE

FDA APPROVED
LEVEL 5 OF 5

- Approved in 2010 for HIV belly fat (Egrifta)
- Two 52-week RCTs, 816 people
- Sleep: one 22-person pilot, no change

NOT YET ESTABLISHED

- Long-term heart safety
- Long-term high IGF-1 effects
- Effect on sleep

HOW IT WORKS IN THE BODY

Tesamorelin ▶ GRF receptor ▶ Growth hormone ▶ IGF-1, belly fat

- **Binds GRF receptors** with potency like natural GRF, in vitro (label)
- **Pituitary releases** the body's own growth hormone, per the label
- **IGF-1 rose** by about 107 ng/mL in the approval trials
- **For sleep,** no effect or mechanism is established

QUICK PRACTICAL INFO

HALF-LIFE	ROUTES STUDIED
11 minutes (single dose under the skin, label)	Under the skin (injection)
PHARMACOKINETICS	**SOLD AS**
Peak in about 0.15 hours; bioavailability under 4%	Prescription vials (Egrifta SV, Egrifta WR)
MONITORED IN TRIALS	**SPORT (WADA)**
IGF-1, blood glucose, eye checks (label)	Prohibited, class S2

REGULATORY

FDA	Approved (Egrifta, 2010) for HIV belly fat only
EMA	Application withdrawn, 2012
WADA	Prohibited (S2.2.4)

REFERENCES

1. FDA, Egrifta WR prescribing information (DailyMed), Mar 2025
2. Stewart et al., eNeurologicalSci, low-dose tesamorelin pilot, Sep 2026
3. WADA, 2026 Prohibited List

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